



Physiotherapy & Rehabilitation Governance Platform

Medical Report: Post-ACL Reconstruction Rehabilitation

Patient Information

Name: ahmet kalumian

Birth Date: 2025-10-28

Case ID: #24

Clinical Details

Doctor: Dr. PSP

Date: 2026-03-28

Description: A 28-year-old male athlete, 4 weeks post-op for ACL reconstruction using a hamstring graft. Currently experiencing mild swelling and limited range of motion (ROM) in the right knee.

Physical Measurements

Height: 182 cm

Weight: 78 kg

Blood Group: OPositive

Health History

Smoker: No

Chronic Diseases: No

Activity Level: Active

Additional Patient Info

InjuryHistory: Complete ACL tear

occurred during a football match on January 15, 2026, due to a sudden pivoting motion. Surgery was performed on March 1, 2026.

Medications: Ibuprofen 400mg as needed for pain, Vitamin D3 supplements, and Omega-3 fish oil.

FunctionalAbility: Partial weight-bearing with one crutch. Knee flexion is currently at 95 degrees. Struggles with stairs and long-distance walking.

PersonalGoals: Full return to competitive football within 9 months, restoration of full quad strength, and achieving 135-degree knee flexion.

Smart Clinical Analysis

Clinical Explanation:

The patient is a 28-year-old male athlete who is currently four weeks post-operative following ACL reconstruction using a hamstring graft. He presents with mild swelling and limited range of motion (ROM) in the right knee, which is common in the early stages of rehabilitation. The patient has a functional ability of partial weight-bearing with the assistance of one crutch and has demonstrated a gradual improvement in knee flexion. His personal goals include a full return to competitive football within nine months, restoration of full quadriceps strength, and achieving 135 degrees of knee flexion.

Immediate Physiotherapy Recommendations:

The focus of physiotherapy should be on gentle range of motion exercises to improve knee flexion, along with strengthening exercises for the quadriceps and hamstrings. Modalities such as ice and elevation may be utilized to manage swelling. Gait training should be emphasized to enhance weight-bearing capabilities and promote proper mechanics during ambulation.

Precautions:

The patient should avoid high-impact activities, sudden pivoting motions, and any exercises that cause pain or discomfort in the knee. Care should be taken to monitor swelling and pain levels during rehabilitation sessions.

Estimated Recovery Timeline:

Given the current progress and the patient's goals, a full recovery with return to competitive football is anticipated within 6 to 9 months, contingent upon continued improvement in knee flexion and strength.

Risk Assessment:

Potential risks include re-injury of the ACL, development of stiffness in the knee joint, and delayed recovery due to excessive swelling or pain. Close monitoring of the patient's response to rehabilitation is essential.

Red Flags:

Emergency warning signs include severe swelling, increased pain not relieved by medication, inability to bear weight on the affected leg, signs of infection (such as fever or redness), and any sudden changes in the knee's stability.

Test Group: Range of Motion (ROM) Assessment

Test: Knee Flexion (Right)

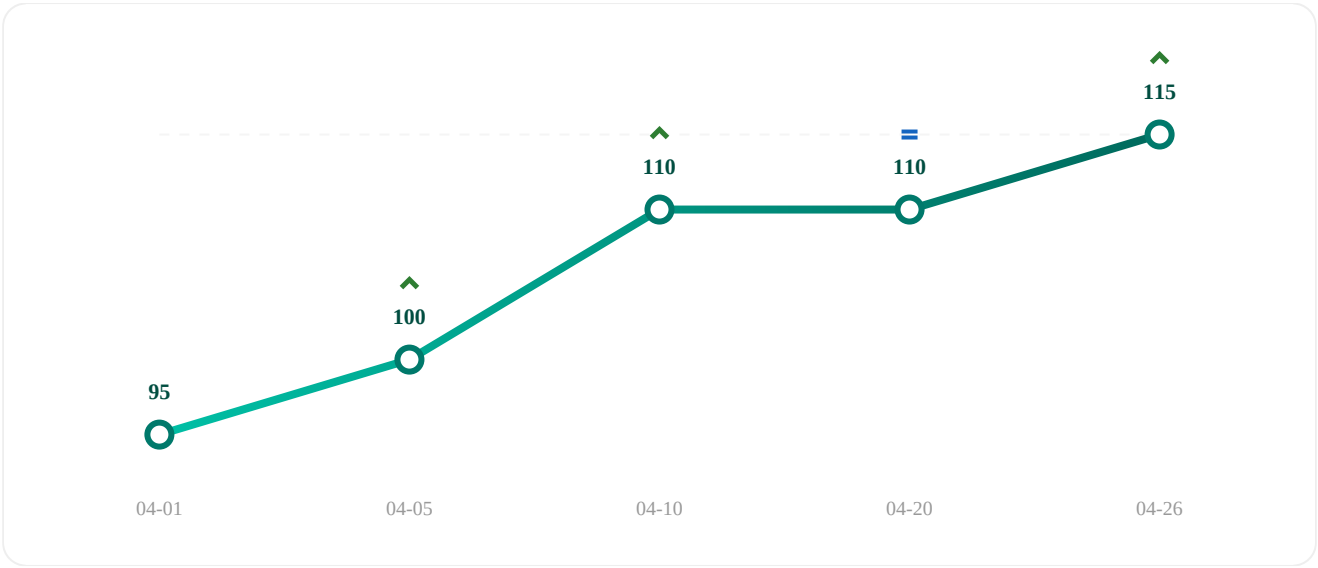
HIGHEST
115.0

LOWEST
95.0

AVERAGE
106.0

OVERALL PROGRESS

Increased by 21.1% ↑ Since first test (2026-04-01)



Date	Result
2026-04-01	95
2026-04-05	100
2026-04-10	110
2026-04-20	110
2026-04-26	115

